

THE

Geriatrician's Guide

A GERIATRICIAN'S FIELD GUIDE

The Tour Day Checklist

The 57 questions a geriatrician would ask — and what the answers really tell you.



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*No placement commissions.
No facility money. Ever.*

FROM THE AUTHOR

If you're reading this, you're probably doing everything right.

You're researching. You've scheduled tours. You're trying to make the best possible decision for someone you love, under pressure, often on a timeline you didn't choose, with information curated by people paid to make every community look like the right one. You may be touring five communities. Maybe ten. You're managing logistics, finances, and the kind of emotional weight that doesn't leave much room for discernment.

None of this is easy. I've watched hundreds of people go through it, and even the most prepared walk out of tours not knowing what they missed — not because they weren't paying attention, but because nobody ever showed them where to look.

That's what this checklist is for.

I'm a geriatrician, which means I've spent my career caring for older adults in the most complex stages of their health. But the experience behind this checklist goes deeper than clinical care. Most physicians have never set foot in a senior living community. The day-to-day reality of these places is invisible to the average doctor — coordinated through fax lines and pharmacy portals, managed by staff with varying levels of clinical training, often without a physician or nurse physically present on any given day. I know that world because I worked inside it, visiting dozens of communities across assisted living and memory care. I met with the nurses and directors responsible for what happened to residents between my visits, reviewed medication lists, and learned the prescribing and pharmacy processes well enough to know exactly where they fail. I've filled out the regulatory paperwork. I've seen where the handoffs break down. I know what a well-run community looks like from the inside, and which warning signs are nearly impossible to spot without that kind of access.

This is your insider's guide. It's organized the way I'd walk a community myself: from the questions worth asking before you schedule the tour, through the front door, across the floor, and into the conversations most people never think to have. You don't have to get this perfect. You just have to walk in knowing where to look — and with this checklist you will.

— Dr. Michael Genualdi



HOW THIS WORKS

One guide to prepare for the day. Three companions to help get through it.

This is the full Tour Day Checklist. It explains the reasoning and the warning signs behind every question worth asking. It's meant to be read in the days before your tour.

THE THREE COMPANIONS

1 The Tour Day Card

A pocket version of every question, compressed to bring on the tour.

2 The Tour Notes

A place to write down what you hear, organized by the same ten sections as the Card.

3 The Community Scorecard

A scoring sheet for comparing communities.

ABOUT THE AUTHOR

The Geriatrician's Guide is founded and edited by **Michael D. Genualdi, MD**, a board-certified geriatrician and internist. After completing his residency and fellowship training at Stanford, he spent years caring for older adults across the full range of care settings — including serving as medical director of a primary care practice embedded in dozens of senior living communities. He built this resource to share what that experience has taught him, so people can care for a loved one with confidence instead of guesswork. The Geriatrician's Guide does not accept compensation of any kind from senior living operators, placement services, or referral networks.

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Before You Go

The tour starts before you walk in. Here's what's worth doing in the day or two beforehand.

Look up the community's inspection and citation history before you schedule. In most states, assisted living inspection reports and citation history are public record. Search for "[your state] assisted living inspection records." You'll likely find them through your state's Department of Social Services, Health, or Aging. You're not after a spotless history; almost no one has one. You're after what and how recent. A medication or staffing citation in the last year or two tells you exactly what to ask about on the tour.

Find out who actually owns and operates the community. The name on the building often isn't the company running it. Many communities belong to large regional or national operators. A growing number are owned by private-equity firms, whose financial model can shape staffing and pricing in ways that affect care. Look up the parent company and what's known about how it runs its other buildings. This isn't "big is bad"; well-run chains and struggling local operators both exist. You're after the track record of whoever's really in charge, since a problem that shows up across many buildings is an ownership problem, not a one-off.

Have your loved one's physician complete the state's health assessment — and read it yourself first. Most states require a physician-completed health assessment at admission. Getting it done early isn't just about speeding up paperwork. Reading it yourself is how you learn what level of care you're actually shopping for, and it arms you with specifics to press each community on whether they can meet *these* needs, not a vague version. A community that engages the specifics, rather than waving them off, is one you can work with.

Decide which sections matter most for your loved one's situation. This is a long list, and not every section earns equal time on every tour. If dementia is in the picture, the memory-care questions are where you spend your time. If mobility is the concern, the environment and accessibility items. If money may run short, read the cost section closely first. Every section may matter at some point in your loved one's stay, but know before you walk in which questions you can't afford to skip today.

Write down two or three questions that feel uncomfortable to ask. The questions people flinch from are usually the ones that matter most: what would get my loved one asked to leave, what the overnight staffing actually looks like, what happens when the money runs out. A good community answers them directly. Writing them down beforehand means you won't lose your nerve in the room.

Arrive a few minutes early, and use them. Get a feel for the neighborhood. Walk around the parking lot, the entrance, watch who comes and goes. The minutes before a scheduled tour are the most unscripted of the visit, before anyone is performing for you. What you notice then is real data.

01 Dining

Visit during an actual mealtime. Watch how long residents wait for food, whether it arrives at a decent temperature, whether staff are attentive, whether people who need help get it without asking twice, and whether residents have any say in what they're eating. The staged dining room and the room during Thursday night dinner are two different places.

If your loved one needs help eating, ask where that happens and who provides it. A separate dining area for residents who need assistance can make logistical sense, but it can also quietly cut someone off from the people they eat with. Ask whether the help comes from community staff or whether you'd be expected to arrange and pay for outside caregivers. That answer tells you about the care model and your future costs.

Ask how medical diets are handled. A strong answer describes a system: how a diabetic, low-sodium, or renal diet gets documented and how it reaches your loved one's plate. A weak one is "we can accommodate that" with nothing behind it. A checkbox on an intake form that no one references in the kitchen is not a system.

Ask how they assist residents who have trouble swallowing. A real dysphagia program produces two things: texture-modified food and thickened liquids, and dining staff who know who needs them without checking. A weak answer is "we can puree things." Ask whether modified meals are appetizing or uniformly pureed, because how much someone eats depends on it.

Ask how staff would notice a resident starting to have trouble swallowing, and what action they take. The dangerous case isn't the diagnosed one — it's the resident who quietly begins coughing at meals, sounding wet or hoarse after drinking, or eating slower and slower. Staff trained to catch and report those signs help mitigate the risk of aspiration (food or liquid entering the lungs), a potentially life-threatening issue.

Ask who catches weight loss, and what happens when they do. Weight loss is one of the earliest signals that something is wrong, but only if someone is looking. A strong answer names who weighs residents, how often, and who notices, documents, and calls you when intake drops. A community without a real answer tends to find out when the change is already visible.

Is a registered dietitian (RD) involved in residents' care? Most communities don't keep one on staff full-time, and unlike nursing homes — where dietitian services are federally required — assisted living and memory care usually aren't required to have one at all. What matters is whether an RD reviews the overall menu and the residents most at risk. Ask how a resident gets flagged for that review and how often it happens. "We have a dietitian on contract" means little if no one triggers the referral.

02 Environment

Look for fall hazards as you walk the halls. A fall and injury can quickly set off a cascade of decline, so read the building for fall risk first. Look for even lighting and nightlights along the paths to bathrooms, handrails secured to hold real weight, and floors that transition between surfaces evenly. A half-inch edge invisible to you is a trip hazard for someone with a shuffling gait or a walker.

Check the areas that aren't on the tour route. Everyone cleans the lobby. A side corridor, a hallway near the laundry, or a bathroom that isn't the designated tour bathroom show you the baseline maintenance standard rather than the performance. Pay attention to odor. A persistent smell of urine isn't just unpleasant; it's a signal about how incontinence is actually being managed day to day.

See the real unit your loved one would live in, not the staged model. Stand in the actual room and look at it as a place someone lives when they're not at their best. Is there room to move with a walker, somewhere to sit that isn't the bed, and can your loved one control the temperature? Spend most of your attention in the bathroom: grab bars where someone actually reaches for them rather than a towel bar that pulls free, a step-free shower entry, room for a helper. A safe and functional bathroom is one of the strongest quiet signals a community understands how its residents live.

Assess how the building works as mobility declines. Mobility tends to move one direction. Ask whether the unit can take a wheelchair, whether there's a backup when the elevator fails, and whether the common spaces your loved one would use every day are accessible without stairs, and navigable with a walker or wheelchair. Even if they don't use an assistive device today, you're planning for who they'll be in the years to come.

Ask about building security. An unstaffed entrance is a gap both ways: anyone can walk in, and a resident inclined to wander can walk out. Look for a staffed or monitored entrance, and ask how after-hours entry and visitor sign-in work. Ask too whether there are security personnel on site, or whether entry is managed by reception and care staff — most communities have no dedicated guards, so a "no" isn't a red flag on its own.

03 Location and Access

Consider whether the people who matter will actually visit. A community an hour from everyone who loves your loved one starts at a deficit no amenity makes up for. Visits are what hold someone to their old life, and distance quietly erodes them. Be honest about who will realistically come, and how often, at this location versus a closer one. Proximity to the family and friends already in the picture often matters more to daily wellbeing than anything on the tour.

03 Location and Access, continued

Ask about transportation to appointments and outings. Most older adults stop driving at some point, so the community becomes the link to the outside world. Ask what's actually provided: scheduled rides to medical appointments, group outings, and whether there's a fee or a limit. A loved one who can't easily get to their doctor, or who's effectively confined to the building, is a real and common outcome that can contribute to isolation and accelerate a person's decline.

Notice the neighborhood, and what's nearby. You got a first read pulling in early; now widen it. Is the area safe and pleasant to walk or sit outside in? Are the doctors your loved one already sees, and the places familiar to them, within reasonable reach? Staying near established physicians and known surroundings eases a transition that's hard enough on its own.

04 Staffing

Ask for the staffing ratio. How many residents is each caregiver responsible for, overnight and on weekends, not just during the day? Most states set no minimum ratio for assisted living, only "sufficient" or "adequate," which means very different things from one building to the next. Ask for the actual numbers — residents per aide on each shift — and whether there's a plan to cover unexpected callouts.

Gauge how quickly staff respond when a resident needs help. Most communities use call buttons as the primary way residents signal for assistance. Ask what their standard is, and what it realistically looks like at 2pm versus 2am. Different answers tell different stories. How long your loved one would have to wait on the floor after a fall, or at the bathroom door, is a direct function of how many people are working that shift.

Determine what clinical training is present on each shift. On any given shift, the most clinically trained person in the building might be a registered nurse, a licensed practical nurse (LPN/LVN), a medication technician, or a caregiver with no clinical license at all. These are meaningfully different levels of training, with different scopes of what each is allowed to assess and do. You're not looking for a nurse on every shift, since most communities don't have one. What you're learning is the ceiling: on a given shift, is the most trained person a licensed nurse, or a medication technician? In memory care especially, where medical and behavioral complexity runs higher, that overnight and weekend ceiling is one of the most important things you can pin down.

Assess caregiver tenure. Turnover in senior living is high, and its consequences are clinical, not just operational. Aides who know your loved one's baseline — how they typically move, how they usually eat, what a good day looks like for them — are the ones who notice when something is off. True one-to-one consistent assignment is rare, but many communities do maintain consistent staffing by floor or wing. Ask whether that's the practice here. A sharper question: how many of the aides on this floor were here a year ago? The answer beats the policy.

04 Staffing, continued

Find out whether shifts are filled by staff or agencies. When a community can't cover its own shifts, it brings in agency or temp workers who don't know the residents. Some use is normal, especially around callouts. But heavy, routine reliance on agency staff undercuts everything continuity depends on — the aide caring for your loved one may be meeting them for the first time, with no sense of their baseline. Ask how often agency staff are used, and whether the same agency people return or it's whoever's available that day.

Learn who runs clinical operations day to day. In most assisted living communities, the person responsible for overseeing your loved one's daily care isn't a physician — it's a director whose title might be Wellness Director, Resident Care Director, or Director of Health Services. This person sets the clinical culture of the building, and while many directors have a nursing background, the job centers on business operations and regulatory compliance in addition to resident services. Ask how long they've been in the role and what their clinical background is. Some states don't require this position to hold a clinical license at all, which is exactly why the question is worth asking. High turnover in this seat is one of the most reliable warning signs I know.

05 Responding to Changing Needs

Ask how emergencies are actually handled: the procedure, start to finish. Senior living communities aren't medical facilities, so when something goes wrong, what matters is whether there's a clear procedure and whether the people on shift know it. Walk it through with them: a resident falls, or suddenly changes — who assesses them first, what's the threshold for calling 911 versus managing in place, and who makes that call? A reflexive trip to the hospital carries real risks for frail older adults, especially ones with cognitive impairment, so you want judgment in that decision, not a default. Finally, when it happens, who calls you, and how fast?

Trace how a routine clinical concern gets escalated. When an aide notices something that isn't an emergency — a change in appetite, new confusion, a wound that isn't healing — ask who they call and how quickly someone responds. With no nurse or physician on site daily at most communities, this routine path reveals the real medical support behind the org chart.

Find out whether any medical care is delivered on site. Most communities don't have this, so treat it as a plus rather than a baseline. If there's an embedded nurse practitioner, physician, or clinic with a regular, scheduled presence — not just on call — ask how often they're there and whether they're internal or an outside group. On-site primary care is one of the few real upside differentiators you'll find.

05 Responding to Changing Needs, continued

Learn about clinical partners: home health, physical therapy, hospice. Communities with established partnerships tend to manage health changes with less disruption to the resident. Providers who work in the building regularly already know the staff, the layout, and how the place runs, so care ramps up faster and fewer details fall through the handoff. Ask who they work with and how long those relationships have run. One thing worth knowing: under the Medicare hospice benefit you may choose any certified hospice, regardless of the community's preferred provider — they cannot require you to use theirs.

Bring the health assessment and review your loved one's needs with the care director. This is where the pre-tour assessment pays off. Walk the director through your loved one's actual documented conditions and watch whether they engage those specifics or retreat to general reassurance. A community that reasons about the real situation in front of them is showing you something a brochure can't.

Ask whether they can care for your loved one as needs rise, and at what point they can't. "We'll work with you" is not an answer. "We can manage help with bathing and dressing, but a two-person transfer is beyond us, and here's what happens then" is. Get the specific threshold, not reassurance. If your loved one has a progressive condition, you already know roughly which way needs will move — use that to press on where their limit actually falls. This question is about staying in assisted living as needs grow; the memory-care section covers progression there.

Ask how the service plan is created and kept current. The service plan is the document that spells out exactly what care your loved one will receive — what help, how often, by whom. You won't get a finished one on a tour; it's built from a formal assessment at or after move-in. So ask the process: who builds it, who's involved, and how often it's revisited as needs change. Because the plan often sets your care tier, a change to it can change your bill — worth keeping in mind alongside the cost questions later. A vague answer about a document you'll "sort out later" is itself a signal.

06 Medications

Ask who administers medications, and who they call when something's unclear. In most communities it's a medication technician, not a nurse. That isn't a problem in itself; it's simply how senior living communities are staffed. What matters is the escalation: when there's a medication question — a missed dose, a reaction, a refusal — who do they reach, and how fast? If your loved one will be managing their own medications, ask how often that ability is reassessed and what would trigger a fresh look. Capacity can change, and a plan that made sense at move-in may not a year later.

06 Medications, continued

Find out how medication changes get tracked. Transitions are where medication errors cluster, and not just hospital discharges: a routine PCP or specialist visit can quietly change a medication list too. A community can't act until a signed order arrives, so ask who obtains it, how fast, and what happens in the gap before it does.

Ask how as-needed (PRN) medications are decided and overseen. A PRN assumes someone can recognize the need — which breaks down for a resident who can't say "I have a headache," so someone has to be watching for it. Ask who makes that call and how. Ask too what PRNs cost, since per-dose fees add up quietly.

Learn about the pharmacy, and what happens when a medication runs out. Most communities use a preferred delivery pharmacy, which helps with consistency and convenience. Ask what it costs, since markups add to the bill, and whether your loved one can use their own instead. Then ask the real question: supply gaps happen more often than you'd expect, so what's the plan when a refill is late?

Determine how often someone reviews the full medication list. Medications accumulate from different prescribers and can interact in ways no one intended. This is polypharmacy, and it's a leading cause of preventable hospitalization in older adults. Some states require a periodic review, but the minimum is usually low — annual is a common floor. Ask what happens between, and whether a pharmacist is ever involved.

07 Activities and Engagement

Look past the activity calendar to what actually runs. A full calendar posted by the elevator means little if half of it gets cancelled or doesn't fit your loved one. You can't verify last month from a tour, so do two things instead: notice whether anything scheduled is actually happening during your visit, and ask a resident or staff member which activities run consistently and if any get cancelled. Routinely cancelled activities are a staffing signal as much as a programming one. Then ask what a real day would look like for someone at your loved one's level of ability and interest.

Ask whether programming spans real range — and includes every ability level. Good programs aren't built on one mode: physical (a walking group, chair exercise), mental (games, discussion, music, art), social, and simply fun all matter. The sharper test is inclusion — does the exercise class or the trivia hour actually involve residents with mobility limits or cognitive impairment, or do they get left at the edge of the room? Inclusive on the calendar isn't the same as inclusive in practice.

07 Activities and Engagement, continued

Determine what staff do when a resident starts to withdraw. The resident who stops coming to meals and activities is the most at risk and the easiest to lose track of — and depression, grief, and anxiety are common and under-recognized in older adults. The question is whether anyone notices early. A strong answer describes how staff flag a resident who's pulling back and who follows up, especially for residents with no loved ones checking in. A weak one treats attendance as optional and finds out something's wrong only when it's already serious.

Learn whether residents have a real say in what's offered. A resident council or similar channel signals a community that treats residents as people with preferences, not occupants on a schedule. Ask whether one exists, how often it meets, and whether anything actually changes because of it.

08 Culture and Communication

Position yourself to see what isn't staged for you. Warmth in front of a touring family is easy to produce. Linger in a common area, let the guide get briefly distracted, take your time in a hallway, and watch the small things: how staff speak to residents who aren't part of your group, and whether they knock and wait before entering a room. A resident's room is legally their private space in most states. Anyone can knock; what tells you something is whether they wait for an answer. These unguarded moments tell you whether respect is policy or culture.

Try to speak with a resident — or a visiting family member — directly, not through a facilitated introduction. Someone who has lived there a year, or who visits a parent every week, knows more about daily life than anyone giving the tour. You don't need clinical questions; even body language when you ask how they like it is informative. A resident may answer carefully — they depend on these people — so a visiting family member, with less at stake, will often be more candid.

Ask to speak with a caregiver without the marketing director present. Front-line staff will often tell you what marketing can't. A few good questions: how long they've worked here, what they'd want a family to know, and what the hardest part of the job is. But the content of the answer matters less than how the community responds to the request itself — comfort with it suggests a place with nothing to hide, while hovering, redirecting, or declining is worth noting.

Find out how to raise concerns, and what happens after. Every resident eventually has a complaint, and how a community handles one reveals its culture under pressure. Ask whether there's a clear grievance process, and whether the person describing it sounds like they've used it or is reciting policy. And know your backstop: every state has a Long-Term Care Ombudsman — an independent, free, confidential advocate, established under federal law — for when the community won't resolve something or you don't trust it to. You're never required to exhaust internal channels before reaching them.

08 Culture and Communication, continued

Learn how the community keeps people informed day to day. This is distinct from raising a problem. Does the community reach out proactively, or do you only hear from them at billing time or in a crisis? A place that treats its residents' loved ones as partners keeps them in the loop before things escalate. This matters most in memory care, where your loved one often can't tell you themselves — proactive updates and regular care conferences may be your only window into how they're really doing.

09 Memory Care

Ask how the unit is secured and what happens when a resident wants to leave. Elopement is among the most serious and most preventable risks in memory care. Ask the specifics: are exterior doors secured, are there delayed-egress devices or alerts on perimeter exits? Then the follow-up most families miss: when a resident keeps trying to leave, what then? A locked door is the backstop, not the safety — in memory care, supervision is the safety, and a secured unit without enough staff isn't a safe one. Ask how many staff are on the unit at a time; locks and devices don't substitute for staffing. Know your loved one's right: when redirection fails, they must be allowed out with staff, not confined or medicated into compliance.

Watch how staff respond to agitation, and ask about their approach to behavior. Agitation in people with dementia is communication: pain, fear, overstimulation, boredom. You're watching for calm redirection and curiosity, and listening for whether medication is the first tool reached for or the last. A unit that reaches first for sedation is managing its own difficulty, not the resident's distress, and that is one of the clearest warning signs in memory care.

Look at what residents are actually doing. For people with dementia, engagement is intervention, not enrichment — meaningful activity is what keeps distress, wandering, and medication use down. A room of residents parked in front of a television while staff cluster off to the side is not a neutral scene. Good programming for people with dementia is purposeful and adapted to cognitive level: engagement for someone with late-stage dementia looks very different from engagement for someone with mild impairment. You want to see residents doing things suited to where they are, with staff among them rather than apart.

Ask whether the unit has a consistent daily structure, and what it actually looks like. Predictable routine isn't simply a preference for people with dementia — it's a clinical tool. Familiar daily rhythms reduce the load on an already-compromised mind, lowering behavioral disturbance and improving quality of life. Ask what a typical day looks like: when meals happen, when activities are scheduled, what the morning routine involves. A unit that can describe a consistent, resident-centered day has built routine into its care. One that offers activities "as residents feel like it" isn't using one of the most effective non-drug tools it has.

09 Memory Care, continued

Walk the unit and notice whether it feels calming or agitating. The design is doing clinical work: even lighting without harsh shadows, low noise, clear sightlines, orientation cues like large-print clocks and signage, and ideally familiar spaces such as a kitchen, a workbench, or a garden. You don't need a specialist's eye. Your loved one will feel what you feel, every day.

Ask what dementia-specific training staff receive, and have them name it. Caring well for people with dementia takes skills general caregiving doesn't, and requirements vary widely by state, with some setting none. A community that takes it seriously can name a recognized program and describe what it covers. A vague answer about general orientation is itself the answer.

Ask whether they can care for your loved one through late-stage dementia. Dementia is progressive, and late stages bring needs — heavier physical care, behavioral complexity, end-of-life support — that go beyond what some memory care units are staffed or licensed to provide. So ask first whether this community can see your loved one through: can they care for someone in late-stage dementia here, and do they have an on-site or partnered relationship with skilled nursing and hospice? If the answer is yes, that continuity is a real advantage. If it's no, ask what triggers a move, where residents typically go, and whether there's an established relationship with that next place. Either way, learn it before you sign, as the moment you need this answer urgently is the wrong time to hear it for the first time.

10 Contracts and Cost

Get the complete fee schedule in writing before you leave. The base monthly rate is rarely the full picture. Community fees, care-tier add-ons, and ancillary charges stack on top, and the gap between the quoted rate and the first full invoice is one of the most common after-the-fact surprises. Ask specifically what is and isn't included.

Learn how the level-of-care assessment works, and what each step up costs. Tiered pricing means the bill grows with needs. Ask who conducts the assessment, what triggers a reassessment, how often it happens, and whether you can appeal a result. A community vague about the criteria is telling you that your costs won't be predictable.

Ask what the move-in fee covers, and what's refundable. Ask the uncomfortable scenarios plainly: what happens if it doesn't work out in the first month, or if your loved one dies shortly after move-in? Refund policies vary widely, and understanding it before you sign is far easier than negotiating it after.

Find out about rate-increase history and any caps, and do the math. Annual increases are standard, and they compound. Ask this community's actual history and whether the contract limits how much rates can rise. Project it several years out before you commit.

10 Contracts and Cost, continued

Ask whether the community accepts Medicaid, and what that actually covers. If there's any chance your loved one's private resources could run out during their stay, this isn't optional. Medicaid for assisted living is complex and varies dramatically by state: most states offer a waiver that can help with the *care* portion, but generally not room and board, which is often the largest cost. These waivers aren't entitlements — enrollment is often capped, waitlists are common, and not every community accepts them. Ask whether this community participates, and under what terms a resident who spends down to Medicaid can stay. A community that accepts Medicaid with a clear policy offers real protection; one that doesn't means running out of money could force a move at the worst possible time.

Confirm whether a long-term care insurance policy exists, and whether it applies here. If your loved one holds a long-term care policy, have it reviewed alongside the residency agreement — coverage terms, what triggers benefits, daily limits, and whether this community and level of care qualify. Policies vary widely, and what's covered isn't always what people assume.

Ask what would cause your loved one to be asked to leave, and get it in writing. Three scenarios are worth naming: care that exceeds what the community is licensed or staffed for, behavior it can't safely manage, and financial exhaustion. Each carries a different timeline. A community that can speak clearly about all three takes its obligations seriously.

Determine what happens to your loved one's spot, and the bill, if they are hospitalized. A hospital stay is common, and what the community does during one varies more than people expect. Ask whether they hold the room, whether you keep paying while it's empty (you usually do, at least partially), and for how long they'll hold it. Then the harder question: can they decline to take your loved one back if the hospital stay changed their needs? A community can treat a hospitalization as grounds to reassess and potentially refuse re-admission, so get the policy in writing before you're managing it from a hospital bedside.

After the Tour

The tour is over. Here's how to make it useful.

On your Tour Notes sheet, write down three things before you start the car. One that gave you confidence, one that gave you pause, one question you forgot to ask. Tours blur together, especially several in a week, and specific observations are far easier to act on than general impressions.

Fill out the Community Scorecard while it's fresh. The Community Scorecard included in this kit is designed for exactly this moment. Print one copy before each tour and complete it the same day — ideally within an hour of leaving. The unanswered questions are as informative as the answered ones; a community that deflected or didn't know the answer to a clinical question has shown you something about its culture. The community with the nicest lobby is not always the right answer. The scorecard exists so you have a better filter than that.

Follow up on anything that felt incomplete. You can call back, ask for written answers to questions handled vaguely in person, and request a second visit at a different hour. A community confident in what you'll find welcomes the scrutiny.

Have an elder-law attorney review the residency agreement before signing. These contracts are long and written to protect the community. An hour with an attorney who specializes in elder law is inexpensive insurance against terms that could cost far more later.

Don't decide alone. If your loved one can take part, their sense of the place deserves real weight. Share notes and Scorecards rather than impressions, since notes are easier to compare across visits.

A NOTE BEFORE YOU GO

Choosing where your loved one will live, and trusting strangers with their daily life, dignity, and safety, is one of the hardest things someone can be asked to do. There is no perfect community. There is no tour that answers every question. There is no checklist that removes the uncertainty from a decision this significant.

What this kit can do is make sure you're asking the right questions, in the right rooms, of the right people, and help you see past the freshly painted lobby and the activities calendar to the signals that actually matter. That's what I've tried to give you here.

If you found it useful, pass it along to someone else who might need it.

— Dr. Michael Genualdi

THE

Geriatrician's Guide

KNOW WHAT COMES NEXT

The Long Road

A Geriatrician's Complete Guide to Senior Care Decisions

By Dr. Michael Genualdi

This kit covers tour day. For everything before and after it — recognizing when it's time, understanding the financial picture, knowing what to watch for after move-in, and having the harder conversations no one prepares you for — I wrote a complete resource. It's the book I wish every family had before they needed it.

[Read The Long Road →](#)



Michael D. Genualdi, MD
Board-Certified in Geriatrics & Internal Medicine

No placement commissions.
No facility money. Ever.